

PATIENT DIET COMPANION · KETOANALOGUE GUIDE

LPD & VLPD Diet Plan for CKD

Meal plans for 40–80 kg patients, Filipino food reference, calorie boosters, and practical adherence tips

— a companion to the Ketoanalogue Supplementation guide.

Last Reviewed: 2026

2026 Edition

KDIGO 2024-aligned

Philippine food context

Print-friendly — A4

HOW TO USE THIS GUIDE

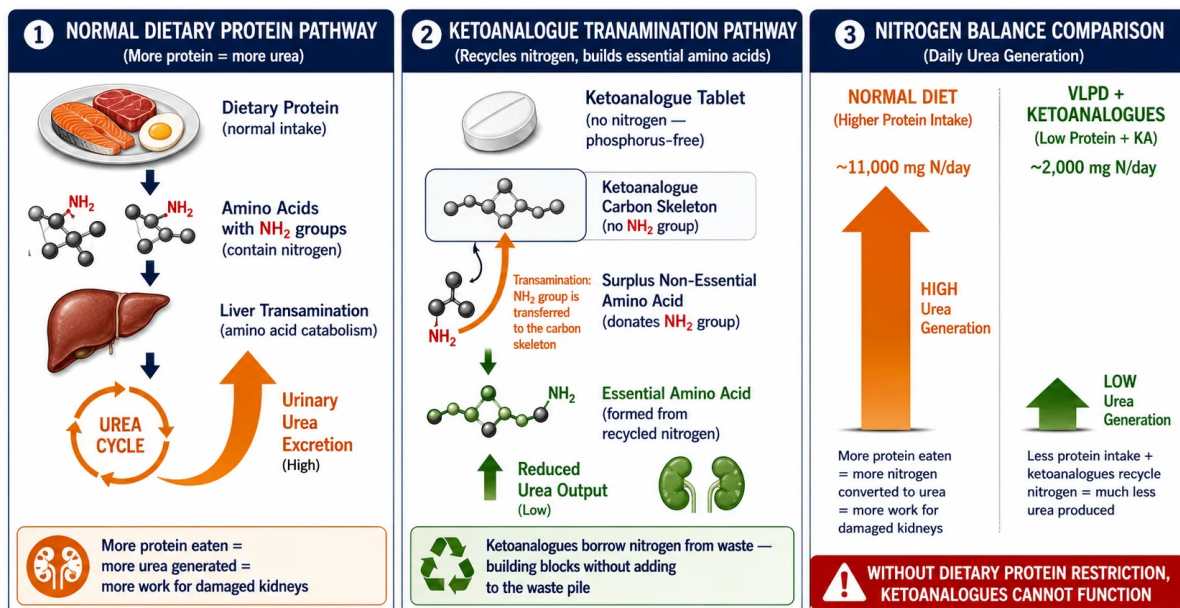
Before You Begin

Ketoanalogues only work alongside a protein-restricted diet. This guide helps you understand exactly how much protein and how many calories you need each day, and gives you realistic Filipino meal examples for your weight class.

Two diet levels are used with ketoanalogues: **LPD** (Low Protein Diet — 0.6 g/kg/day, used in CKD stage 3b–4) and **VLPD** (Very Low Protein Diet — 0.3 g/kg/day, used in CKD stage 4–5 when KA are prescribed). Always confirm your diet type and daily targets with your nephrologist before starting.

HOW KETOANALOGUES WORK

The Transamination Mechanism — Why Diet Is Non-Negotiable



KEY TAKEAWAY: Ketoanalogues provide carbon skeletons. They **ACCEPT** nitrogen from non-essential amino acids to **BUILD** essential amino acids — reducing urea production when protein intake is restricted.

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Fig. 1 — How ketoanalogues recycle nitrogen instead of adding to the urea burden. This mechanism only works when dietary protein is simultaneously restricted.

Normal Diet (No Restriction)

When you eat a normal Filipino diet — rice, meat, fish, eggs freely — your body processes roughly **70–80 g of protein per day**. Each gram of protein contains about 160 mg of nitrogen. When broken down, this nitrogen becomes **urea**, which damaged kidneys struggle to excrete. Blood urea nitrogen (BUN) rises, worsening uremic symptoms. Ketoanalogues taken on top of this diet are biochemically overwhelmed — they can only recycle ~432 mg of nitrogen per day (12 tablets), compared to the ~11,000 mg generated by a normal Filipino diet. The drug is clinically useless and the calcium load accumulates.

VLPD + Ketoanalogues (Correct Use)

On a very-low-protein diet (0.3 g/kg/day), you restrict dietary nitrogen to a fraction of normal. Ketoanalogues then act as **nitrogen-free amino acid precursors** — their carbon skeletons accept nitrogen recycled from your body's own surplus non-essential amino acids through transamination. This builds the essential amino acids your body needs without adding new nitrogen. BUN falls, uremic toxin load decreases, and kidney function decline slows. This is the mechanism proven in the landmark Garneata trial — but it **only works at 0.3 g/kg/day protein restriction, not on a normal diet**.

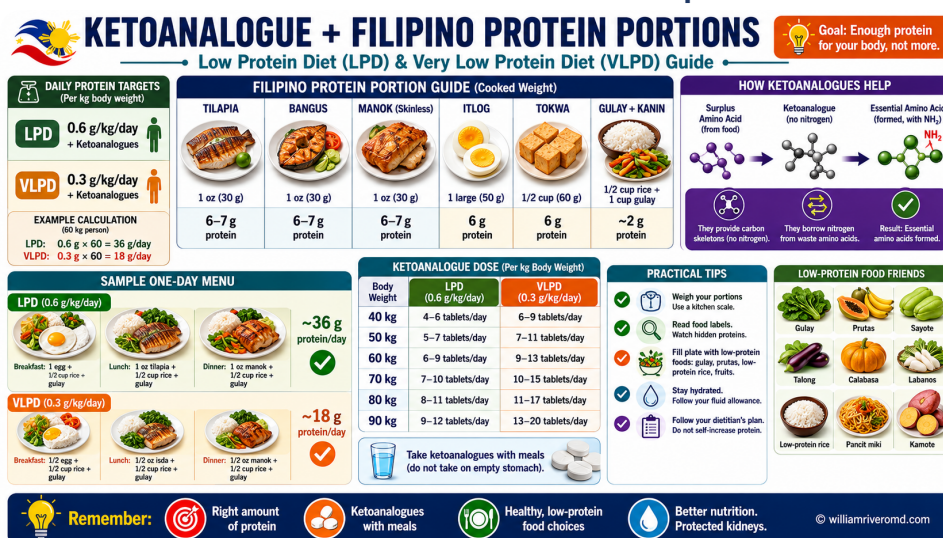
QUICK REFERENCE

Your Daily Targets by Body Weight

Body Weight	LPD Protein	VLPD Protein	Calories	KA Tablets
	0.6 g/kg/day	0.3 g/kg/day	30–35 kcal/kg	1 tab per 5 kg
40 kg	24 g/day	12 g/day	1,200–1,400 kcal	8 tabs
50 kg	30 g/day	15 g/day	1,500–1,750 kcal	10 tabs
60 kg	36 g/day	18 g/day	1,800–2,100 kcal	12 tabs
70 kg	42 g/day	21 g/day	2,100–2,450 kcal	14 tabs
80 kg	48 g/day	24 g/day	2,400–2,800 kcal	16 tabs

Calorie target is NOT optional. If you eat too few calories, your body burns amino acids (including those from KA tablets) for energy — making the ketoanalogues useless. You must hit your calorie target every day using rice, kamote, cassava, oils, and other non-protein foods. See the Calorie Booster section on page 4.

VISUAL REFERENCE

What LPD and VLPD Look Like on a Filipino Plate**The bottom line:**

Ketoanalogues are the lock. Dietary protein restriction is the key. Without the key, the lock never opens — but the calcium from each tablet still accumulates daily, causing net harm.

Fig. 2 — Filipino-contextualized protein portions for LPD and VLPD, with calorie compensation sources. This is what your daily protein allowance looks like in real food — far smaller than a normal Filipino meal.

LPD — 0.6 g/kg/day (e.g. 36 g for 60 kg)

On LPD, your total daily protein from all food sources is roughly the equivalent of **1 medium egg + 1 small bangus fillet (60 g) + a small serving of tofu** — spread across the entire day. This already includes the background protein from 3 cups of rice (~12 g). What you cannot eat: additional meat, pork, chicken, dairy, processed foods, or large portions of legumes. What fills the rest of your plate: rice, root crops, vegetables, and cooking oil.

- 1 whole egg at breakfast
- 60 g bangus or tilapia at lunch
- 50 g tofu at dinner
- 3 cups rice throughout the day (~12 g background protein)
- No additional meat, dairy, or processed proteins

VLPD — 0.3 g/kg/day (e.g. 18 g for 60 kg)

VLPD is twice as restrictive as LPD. For a 60 kg patient, 18 g of total daily protein means roughly **1 whole egg** from protein foods — because 3 cups of rice already contribute ~12 g, leaving only ~6 g for actual protein dishes. In practice this means a single egg, or a very small cube of tofu, is your entire protein food allowance for the day. The rest of your plate is almost entirely rice, kamote, cassava, gabi, vegetables, and oil. This is the diet studied in the Garneata trial that demonstrated delayed dialysis.

- 1 whole egg (total protein food for the day)
- 3 cups rice (~12 g background protein)
- Kamote, cassava, gabi for extra calories (low protein)
- Generous oil in every dish for calorie density
- No fish, meat, dairy, nuts — severely restricted

PART A

LPD Meal Plans — 0.6 g of Protein per kg per Day

Who follows LPD? Patients with CKD stage 3b–4 (eGFR 15–44), where protein restriction alone may stabilize kidney function. KA are optional at this stage; some patients progress to VLPD + KA when eGFR drops below 25. On LPD, you are allowed moderate amounts of high-quality protein — but far less than a typical Filipino diet. **Rice contributes ~4 g of protein per cup** — always count this toward your daily budget. Use egg whites, small fish portions, and tofu as your preferred protein sources. Approximate values — actual intake may vary ±3 g protein per day. A dietitian consultation is strongly recommended.

40 kg Patient — LPD 0.6 g/kg · 24 g protein/day · 1,200–1,400 kcal/day
24g
Protein
1,300
kcal target
8
KA tabs

Meal	Foods	Protein ~kcal	
Breakfast 7 AM	1 whole egg (scrambled in 1 tsp oil) + ¾ cup cooked rice + 1 cup sautéed sayote in oil + 2 KA tablets Rice is your primary calorie base. Use at least 1 tsp oil per dish.	~9 g	~340
Lunch 12 PM	30 g steamed bangus (palm-width, ½ cm thick) + 1 cup cooked rice + boiled pechay with oil + 3 KA tablets Bangus portion = roughly the size of 2 fingers. No added salt — use calamansi for flavor.	~10 g	~295

Calorie compensation is mandatory.

Both diets require you to eat more carbohydrates and fats than a normal Filipino diet — not less — to prevent muscle wasting. The goal is adequate energy from non-protein sources so the body does not break down muscle for fuel.

Meal	Foods	Protein ~g	~kcal
Snack 3 PM	1 medium boiled kamote + ½ cup sago in sugar syrup + 1 banana No protein food at snack time — use this meal purely for calories.	~3 g	~305
Dinner 7 PM	50 g firm tofu (fried in oil) + ¾ cup cooked rice + sautéed kangkong in oil + 3 KA tablets Rinse tofu before cooking to reduce potassium. Frying in oil adds needed calories.	~9 g	~330
Daily Total		~ 31 g*	~ 1,270

Calorie gap? Add 1–2 tbsp coconut or canola oil to any dish, or have an extra cup of cassava/gabi. *Protein slightly above target due to vegetable background — common and acceptable; prioritize adequate calories.

50 kg Patient — LPD 0.6 g/kg · 30 g protein/day · 1,500–1,750 kcal/day

30g

Protein

1,600

kcal target

10

KA tabs

Meal	Foods	Protein ~g	~kcal
Breakfast 7 AM	1 whole egg (fried in oil) + 1 cup cooked rice + 1 cup sautéed ampalaya with oil + 2 KA tablets	~10 g	~375
Lunch 12 PM	40 g tilapia (steamed with ginger) + 1 cup cooked rice + boiled sayote + 4 KA tablets Tilapia portion ≈ size of a matchbox. Eat with ½ cup soup (no meat broth).	~12 g	~330
Snack 3 PM	1 cup boiled gabi + 1 ripe saba banana + 1 cup sugar cane juice (no ice) Gabi is an excellent low-protein calorie source — use it to fill calorie gaps.	~4 g	~375
Dinner 7 PM	75 g firm tofu (tokwa't gulay) + 1 cup cooked rice + sautéed pechay in oil + 4 KA tablets	~10 g	~370
Daily Total		~ 36 g*	~ 1,450

Still 100–300 kcal short? Add an evening snack: 1 cup sago + 1 tbsp sugar, or ½ cup sweet corn. Increase cooking oil to 1–2 tbsp per dish.

60 kg Patient — LPD 0.6 g/kg · 36 g protein/day · 1,800–2,100 kcal/day

36g

Protein

1,950

kcal target

12

KA tabs

Meal	Foods	Protein ~g	~kcal
Breakfast 7 AM	2 egg whites + 1 egg yolk (scrambled) + 1 cup cooked rice + 1 medium kamote + sautéed kangkong with 1 tbsp oil + 3 KA tablets Using 2 whites + 1 yolk reduces saturated fat while keeping protein.	~13 g	~440
Lunch 12 PM	60 g bangus (baked/steamed) + 1½ cups cooked rice + boiled sayote + 1 tsp coconut oil + 4 KA tablets This is the reference case in all clinical trials — 60 g bangus portion is about the size of your palm.	~16 g	~520

Meal	Foods	Protein ~g	kcal
Snack 3 PM	1 cup boiled cassava + 1 ripe banana + 1 cup coconut water (no sugar added)	~3 g	~320
Dinner 7 PM	50 g tokwa (fried or adobo-style) + 1 cup cooked rice + sautéed pechay + 5 KA tablets KA tablets: distribute throughout the day with meals, never on an empty stomach.	~9 g	~350
Daily Total		~ 41 g*	~ 1,630

Calorie shortfall is common in Filipino LPD patients. Add an extra cup of rice at lunch, or include 1 tbsp oil per dish. If weight is dropping despite eating well, increase cooking oil portion.

70 kg Patient — LPD 0.6 g/kg · 42 g protein/day · 2,100–2,450 kcal/day

42g

Protein

2,275

kcal target

14

KA tabs

Meal	Foods	Protein ~g	kcal
Breakfast 7 AM	2 whole eggs (sinangag-style with garlic + 1 tbsp oil) + 1 cup cooked rice + 1 medium kamote + 4 KA tablets	~14 g	~490
Lunch 12 PM	70 g tilapia (paksiw or steamed) + 1½ cups cooked rice + boiled upo or labanos + 1 tbsp canola oil + 5 KA tablets	~18 g	~540
Snack 3 PM	1 cup boiled gabi + 1 ripe mango (medium, no skin) + 1 cup pandan sago	~4 g	~430
Dinner 7 PM	100 g firm tofu (ginisang tofu with vegetables) + 1 cup cooked rice + sautéed kangkong + 1 tbsp oil + 5 KA tablets	~12 g	~420
Daily Total		~ 48 g*	~ 1,880

Still ~400 kcal short of target. Add an evening snack (½ cup cassava + sago, or corn + coconut milk/no dairy) and increase oil portions at all 3 meals. 70 kg patients commonly need 4 full cups of rice to reach calorie goals.

80 kg Patient — LPD 0.6 g/kg · 48 g protein/day · 2,400–2,800 kcal/day

48g

Protein

2,600

kcal target

16

KA tabs

Meal	Foods	Protein ~g	kcal
Breakfast 7 AM	2 whole eggs (tortang talong with 1 tbsp oil) + 1½ cups cooked rice + boiled kamote (1 medium) + 4 KA tablets	~16 g	~570
Lunch 12 PM	80 g bangus (sinigang without pork, with kangkong & okra) + 1½ cups rice + 1 tbsp canola oil + 5 KA tablets For sinigang: use sampalok base, no patis — use calamansi to taste.	~20 g	~590
Snack 3–4 PM	1 cup boiled cassava + 2 tbsp coconut cream (not milk) + 1 cup sago	~3 g	~490

Meal	Foods	Protein ~g	kcal
Dinner 7 PM	100 g tokwa (adobo-style, tofu + vinegar + garlic) + 1 cup rice + boiled upo + 1 tbsp oil + 7 KA tablets	~13 g	~460
Daily Total		≈ 52 g*	≈ 2,110

80 kg patients have the largest calorie gap to fill. Target is 2,400–2,800 kcal. Add a morning snack (corn + oil), increase rice to 5 cups total, and use 2 tbsp oil per cooked dish. Without adequate calories, KA and the restrictive diet will cause muscle wasting.



PART B

VLPD Meal Plans — 0.3 g of Protein per kg per Day

Who follows VLPD? Patients with CKD stage 4–5 (eGFR <30), where ketoanalogues are formally prescribed. This is the diet studied in major clinical trials (Garneata 2016). It is extremely restrictive — roughly one-sixth of a normal Filipino protein intake.

VLPD is mostly carbohydrates and fats. You will eat a very small amount of protein food (about 1 egg, or a tiny piece of fish) per day. The rest of your diet is rice, root crops, vegetables, and oil. This feels very different from normal Filipino meals — preparation and mindset adjustment take time.

Important: On VLPD, even rice contributes significantly. Replacing some rice with lower-protein alternatives like cassava, gabi, or kamote helps stay within your protein budget while still getting enough calories.

Approximate values — actual intake may vary ±2 g protein per day. A renal dietitian consultation is essential for VLPD.

40 kg Patient — VLPD 0.3 g/kg · 12 g protein/day · 1,200–1,400 kcal/day · 8 KA tabs

12g

Protein

1,300

kcal target

8

KA tabs

Meal	Foods	Protein ~g	kcal
Breakfast 7 AM	1 egg white only (no yolk) scrambled in 1 tsp oil + ¾ cup cooked rice + 1 medium boiled kamote + 2 KA tablets Only the white — yolk adds 2.7 g protein, which is significant on 12 g/day budget.	~7 g	~315
Lunch 12 PM	¾ cup cooked rice + 1 cup boiled gabi + sautéed sayote with 1 tbsp oil + 3 KA tablets No protein food at lunch — all protein budget was used at breakfast.	~3 g	~370
Snack 3 PM	1 cup sago in sugar + 1 ripe banana	~1 g	~280
Dinner 7 PM	25 g firm tofu (lightly fried) + ½ cup cooked rice + boiled kangkong + 1 tsp oil + 3 KA tablets 25 g tofu = roughly a 2×2 inch cube. This is a very small portion — it is expected.	~4 g	~245
Daily Total		≈ 15 g*	≈ 1,210

VLPD at 40 kg is the most restrictive plan. 12 g/day means almost no protein food — just 1 egg white and a small cube of tofu. Replace rice servings with cassava and kamote to reduce background protein while maintaining calories.

50 kg Patient — VLPD 0.3 g/kg · 15 g protein/day · 1,500–1,750 kcal/day · 10 KA tabs

15g

Protein

1,625

kcal target

10

KA tabs

Meal	Foods	Protein	~kcal
Breakfast 7 AM	1 whole egg (poached or soft-boiled) + ½ cup cooked rice + 1 medium kamote + 1 cup sautéed ampalaya with 1 tbsp oil + 3 KA tablets	~9 g	~415
Lunch 12 PM	¾ cup cooked rice + 1 cup boiled gabi + boiled okra + 1 tbsp canola oil drizzle + 3 KA tablets Flavor with calamansi, garlic, and vinegar — not soy sauce or patis (high sodium).	~4 g	~430
Snack 3 PM	1 cup cassava cake (plain, no coconut milk/cheese) + 1 cup sugarcane juice	~1 g	~360
Dinner 7 PM	25 g firm tofu (fried) + ½ cup cooked rice + sautéed sayote + 1 tsp oil + 4 KA tablets Total for the day: 1 egg + a tiny cube of tofu = all protein food allowed.	~4 g	~290
Daily Total		≈ 18 g*	≈ 1,495

Still ~200 kcal short. Add an evening snack of boiled corn (½ cob) or a cup of sweetened sago/gulaman.

Increase oil in lunch and dinner by 1 tsp each.

60 kg Patient — VLPD 0.3 g/kg · 18 g protein/day · 1,800–2,100 kcal/day · 12 KA tabs

18g

Protein

1,950

kcal target

12

KA tabs

Meal	Foods	Protein	~kcal
Breakfast 7 AM	1 whole egg (sunny side up, 1 tbsp oil) + 1 cup cooked rice + 1 medium kamote + 3 KA tablets This is the landmark trial meal pattern — 1 egg total for the day is the protein centerpiece.	~10 g	~475
Lunch 12 PM	1 cup cooked rice + 1 cup boiled cassava + sautéed sayote and kangkong in 1 tbsp oil + 4 KA tablets Cassava has only ~1 g protein per cup — excellent calorie filler with minimal protein impact.	~5 g	~490
Snack 3 PM	1 cup boiled gabi + 1 ripe banana + 1 cup sago in buko juice	~3 g	~400
Dinner 7 PM	25 g tokwa (fried) + ¾ cup cooked rice + boiled pechay + 1 tsp oil + 5 KA tablets Dinner protein is minimal — just a small cube of tofu to close the protein gap.	~5 g	~310
Daily Total		≈ 23 g*	≈ 1,675

Reference-weight VLPD. Most clinical trial data is based on patients near 60 kg. This is the most studied pattern. Still ~275–425 kcal short — add cooking oil generously (2 tbsp per cooked dish) and include an evening sago snack.

70 kg Patient — VLPD 0.3 g/kg · 21 g protein/day · 2,100–2,450 kcal/day · 14 KA tabs

21g

Protein

2,275

kcal target

14

KA tabs

Meal	Foods	Protein ~g	~kcal
Breakfast 7 AM	1 whole egg (scrambled with 1 tbsp oil) + 1 cup rice + 1 cup boiled gabi + sautéed ampalaya + 4 KA tablets	~11 g	~545
Lunch 12 PM	1 cup cooked rice + 1 cup boiled cassava + 1 cup boiled sayote + 1 tbsp canola oil + 4 KA tablets	~5 g	~490
Snack 3 PM	1 cup sweet corn (no butter) + 1 cup sago in sugar syrup + 1 banana	~4 g	~480
Dinner 7 PM	50 g firm tofu (tokwa adobo) + ¾ cup cooked rice + boiled kangkong + 1 tsp oil + 6 KA tablets 50 g tofu = about 4 g protein. Combined with background protein from rice and vegetables, this stays near the 21 g target.	~6 g	~350
Daily Total		~ 26 g*	~ 1,865

~400–580 kcal gap is large. At 70 kg VLPD, calorie-filling is the biggest challenge. Add 2 tbsp oil to every cooked dish, include a morning snack (sago or cassava), and consider an evening snack of gulaman in sugar syrup.

80 kg Patient — VLPD 0.3 g/kg · 24 g protein/day · 2,400–2,800 kcal/day · 16 KA tabs

24g

Protein

2,600

kcal target

16

KA tabs

Meal	Foods	Protein ~g	~kcal
Breakfast 7 AM	1 whole egg (poached) + 1 cup rice + 1 large kamote (boiled) + sautéed pechay with 1 tbsp oil + 4 KA tablets	~12 g	~570
Lunch 12 PM	1 cup cooked rice + 1 cup boiled gabi + ½ cup boiled cassava + sautéed sayote with 2 tbsp oil + 5 KA tablets	~6 g	~560
Snack 3–4 PM	1 cup sweet corn + 1 cup sago in sugar syrup + 1 ripe mango (medium)	~5 g	~510
Dinner 7 PM	50 g firm tofu (fried in 1 tbsp oil) + 1 cup cooked rice + boiled kangkong + 7 KA tablets Divide KA tablets: 4 at breakfast, 5 at lunch, 7 at dinner = 16 total.	~7 g	~440
Daily Total		~ 30 g*	~ 2,080

80 kg VLPD is extremely difficult to manage without a dietitian. The calorie gap (~500–700 kcal) requires deliberate additions of oil-rich foods, extra snacks, and possibly renal-formula supplements. Morning snack (boiled corn + 1 tbsp coconut cream) and evening snack (1 cup gulaman + ½ cup coconut milk) should be added daily.

PART C

Filipino Food Protein Reference

Food Item	Serving Size	~Protein	~Calories
Protein Foods — Count These Carefully			
Whole egg	1 large (50 g)	6 g	70 kcal
Egg white only	1 white (~30 g)	3.6 g	17 kcal
Egg yolk only	1 yolk (~18 g)	2.7 g	55 kcal

Food Item	Serving Size	~Protein	~Calories
Bangus (milkfish), cooked	60 g (palm-size)	12 g	90 kcal
Tilapia, cooked	60 g	12 g	80 kcal
Bangus, cooked	30 g (half-portion)	6 g	45 kcal
Firm tofu (tokwa)	100 g (~½ block)	8 g	76 kcal
Firm tofu (tokwa)	50 g (~¼ block)	4 g	38 kcal
Firm tofu (tokwa)	25 g (small cube)	2 g	19 kcal
Monggo (mung beans), cooked	½ cup (100 g)	4 g	105 kcal
Monggo (mung beans), cooked	1 cup (200 g)	8 g	210 kcal
Staple Carbohydrates — Background Protein (Count All)			
Cooked white rice	1 cup (186 g)	4 g	200 kcal
Cooked white rice	¾ cup	3 g	150 kcal
Kamote (sweet potato), boiled	1 medium (100 g)	2 g	100 kcal
Gabi (taro), boiled	1 cup (132 g)	2 g	185 kcal
Cassava, boiled	1 cup (206 g)	1 g	165 kcal
Corn (sweet), boiled	1 cup (154 g)	3 g	145 kcal
Sago (cooked in water)	1 cup	0 g	180 kcal
Banana (ripe saba)	1 medium (80 g)	1 g	90 kcal
Vegetables — Small Background Protein			
Kangkong, cooked	1 cup	2 g	25 kcal
Sayote, cooked	1 cup	1 g	25 kcal
Pechay, cooked	1 cup	1 g	18 kcal
Ampalaya, cooked	1 cup	1 g	20 kcal
Okra, boiled	1 cup	2 g	35 kcal
Fats & Oils — Zero Protein, Essential for Calories			
Coconut oil	1 tbsp (13 g)	0 g	120 kcal
Canola oil	1 tbsp (14 g)	0 g	120 kcal
Coconut cream (kakang gata)	2 tbsp (30 g)	0.5 g	100 kcal

Link List

Cocoa Protein

• Dietary Choices

— VLPD

moderate/

protein PD

only Egg

protein

significantly

to very high

K⁺ protein

and protein availability,

protein

phosphorus

- **Whole processed**
eggs
— → **Bangus /**
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PART D

Calorie Booster Foods — Zero or Near-Zero Protein

These foods add essential calories without significantly increasing your protein budget. Use them generously to reach your daily calorie target.

Cooking Oils

Coconut oil, canola — 120 kcal per tablespoon. Add to every cooked dish. 2–3 tbsp/day is essential.

Kamote

100 kcal per medium piece, only 2 g protein. Better than rice for staying within protein limits.

Cassava

165 kcal per cup, only 1 g protein. Boiled, steamed, or as puto. Best low-protein starch.

Gabi (Taro)

185 kcal per cup, 2 g protein. Versatile — boiled, in sinigang, or as binagoongang gabi (no meat).

Sago

180 kcal per cup (cooked), essentially zero protein. Sago gulaman, sago't gulaman in sugar syrup — safe daily.

Ripe Banana

90 kcal, only 1 g protein. But watch potassium — limit to 1 per day if potassium is elevated.

Coconut Cream

~100 kcal per 2 tbsp, very low protein. Add to cassava or sago dishes. Not coconut milk (slightly more protein).

Plain Sugar / Syrup

16 kcal per tsp, zero protein. Used in sago, gulaman, and buko drinks to add calories safely.

PART E

10 Practical Adherence Tips

The most common reason ketoanalogues fail to work is dietary non-adherence — patients take the tablets but don't restrict protein. These tips address the real-world barriers Filipino patients face.

1

Take KA tablets with meals — never on an empty stomach

Ketoanalogues need food present to undergo transamination. Taking them between meals reduces their effectiveness. Use a pill organizer and tie each dose to a meal habit: "I take my tablet when I sit down to eat."

2

Cook your protein food separately from the family's portion

You don't have to cook a completely different meal. Make one pot of rice and vegetables, then add a separate small protein portion (your bangus, egg, or tofu) to only your plate. This is less isolating and easier to sustain long-term.

3

Replace one cup of rice with cassava or kamote every day

This simple swap reduces background protein by 3–4 g per day while adding equivalent calories. Over time it meaningfully helps you stay within your budget. Buy extra kamote — it is cheap, filling, and almost protein-free.

4

Add 1–2 tablespoons of cooking oil to every dish

The single biggest reason patients fail to reach their calorie targets is low fat intake. Filipino diets are often lean — but on LPD/VLPD, you must deliberately add oil to prevent muscle breakdown. Coconut oil and canola oil are both acceptable; lard and margarine are discouraged.

5

Weigh yourself every morning — act if you lose more than 1 kg in a week

Rapid weight loss on a protein-restricted diet almost always means you are not eating enough calories. Your body is burning muscle for energy. Contact your nephrologist or dietitian immediately if you lose more than 1 kg in a week — do not wait for your next scheduled visit.

6

Plan your "protein moments" for the day the night before

With only 1–2 protein portions allowed per day, deciding when to eat them matters. Most patients find it easiest to eat their main protein food at breakfast (egg) — this removes the temptation to "save" it for a social meal and then exceed the budget later in the day.

7

Learn to say "I can't eat that — kidney diet" at social events

Filipino culture centers food in every social gathering. You will be pressured to eat lechon, kare-kare, and processed meats at fiestas and celebrations. Prepare a short, confident explanation: "I'm on a special kidney diet — I can eat the rice and vegetables, but not the meat." Practice this phrase. People respect health restrictions when explained clearly.

8

Track BUN (blood urea nitrogen) — let the numbers motivate you

If you are sticking to the diet, your BUN should fall over 4–8 weeks. Ask your doctor to show you the trend. A declining BUN is direct proof that the diet is working — this is one of the most powerful motivators for continued adherence. If BUN is not falling, you are likely eating more protein than you realize.

9

Have a weekly "diet audit" with your family

Once a week, sit down with the family member who cooks and review the past week's meals. Were oils added consistently? Was the protein portion kept small? Did you skip any KA tablets? Brief weekly reviews catch slippage early, before it shows up in lab results 3 months later.

10

If you slip, restart at the next meal — not next Monday

At a family gathering, you may eat more protein than allowed. This happens. The mistake is compounding it by thinking "I already broke the diet so I'll restart next week." Get back to the plan at your very next meal. One-day slips have a small effect on kidney function; a week of abandonment can significantly increase nitrogen load and BUN.

PART F

Safety Reminders While on KA + Protein Restriction**Seek Medical Attention If You Experience:**

- **Nausea, vomiting, constipation, or excessive thirst** — early signs of hypercalcemia (high blood calcium from KA tablets). Do not ignore; these are not normal side effects to tolerate.
- **Confusion, drowsiness, or muscle weakness** — severe hypercalcemia or protein-calorie malnutrition. Contact your nephrologist same day.
- **Unintentional weight loss of >2 kg in one month** — not enough calories. Your diet needs urgent adjustment before muscle mass is lost.
- **Bone pain, joint pain, or eye redness/blurring** — calcium deposits from prolonged hypercalcemia. Requires immediate evaluation.
- **Your BUN is rising despite taking KA correctly** — almost always means dietary protein is not restricted enough. Do not increase KA dose; review protein intake with your nephrologist.

Never take calcium-containing antacids or calcium supplements while on KA without checking with your nephrologist first. Each KA tablet already contains calcium. Adding more from antacids (Tums, Kremil-S with CaCO_3) or supplements significantly increases hypercalcemia risk.

This diet plan is a companion to the ketoanalogue supplementation guide at williamriveromd.com/guides/ketoanalogue-supplementation.

All protein values are approximate. Consult a registered renal dietitian for an individualized meal plan.

Reference: Garneata & Mircescu, JASN 2016; KDIGO 2024 CKD Nutrition Guideline Update.